

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: SallenPeriod: February 6 to March 7, 2024Report Date: March 8, 2024Coverage: 502/735h (68%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

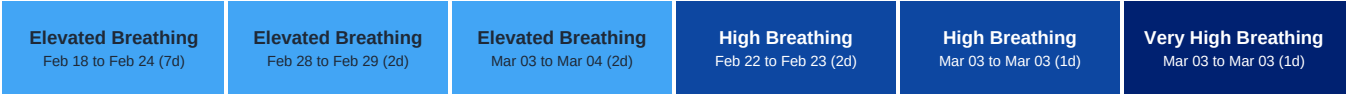
High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 31 days from Feb 06 to Mar 07

HR Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

| Vital      | Avg     | Min    | Max     |
|------------|---------|--------|---------|
| Heart Rate | 58 bpm  | 46 bpm | 83 bpm  |
| Breathing  | 18 brpm | 7 brpm | 44 brpm |

Limited data in the final 24h (17/24h); values reflect available readings.

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

22 episodic events Detected, spanning 23h total. Conditions: Elevated Breathing, High Breathing, Very High Breathing. Priority grade: Low

| Episodic Events | Total Hours | Highest Burden     | Episodes/day | Coverage |
|-----------------|-------------|--------------------|--------------|----------|
| 22              | 23h         | Elevated Breathing | <1           | 68%      |

Major Findings: Stable baseline

| Date   | Time Span      | Duration | Episodes | Condition           | Avg | Min | Max | Comment                                                                  |
|--------|----------------|----------|----------|---------------------|-----|-----|-----|--------------------------------------------------------------------------|
| Mar 03 | 4 PM to 5 PM   | 1h       | 1        | Very High Breathing | 41  | 28  | 44  | Check SpO2, work of breathing, fever; consider pulmonary cause or CHF    |
| Feb 22 | 3 PM to 4 PM   | 2h       | 2        | High Breathing      | 31  | 22  | 43  | Check SpO2, work of breathing, fever; consider pulmonary cause or CHF    |
| Mar 03 | 10 PM to 11 PM | 1h       | 1        | High Breathing      | 31  | 13  | 45  | Check SpO2, work of breathing, fever; consider pulmonary cause or CHF    |
| Feb 18 | 7 PM to 8 PM   | 8h       | 8        | Elevated Breathing  | 27  | 9   | 57  | Check SpO2 and lung sounds; assess for fluid overload or early infection |
| Feb 28 | 10 PM to 11 PM | 6h       | 5        | Elevated Breathing  | 27  | 9   | 54  | Check SpO2 and lung sounds; assess for fluid overload or early infection |
| Mar 03 | 3 PM to 4 PM   | 2h       | 2        | Elevated Breathing  | 26  | 16  | 43  | Check SpO2 and lung sounds; assess for fluid overload or early infection |

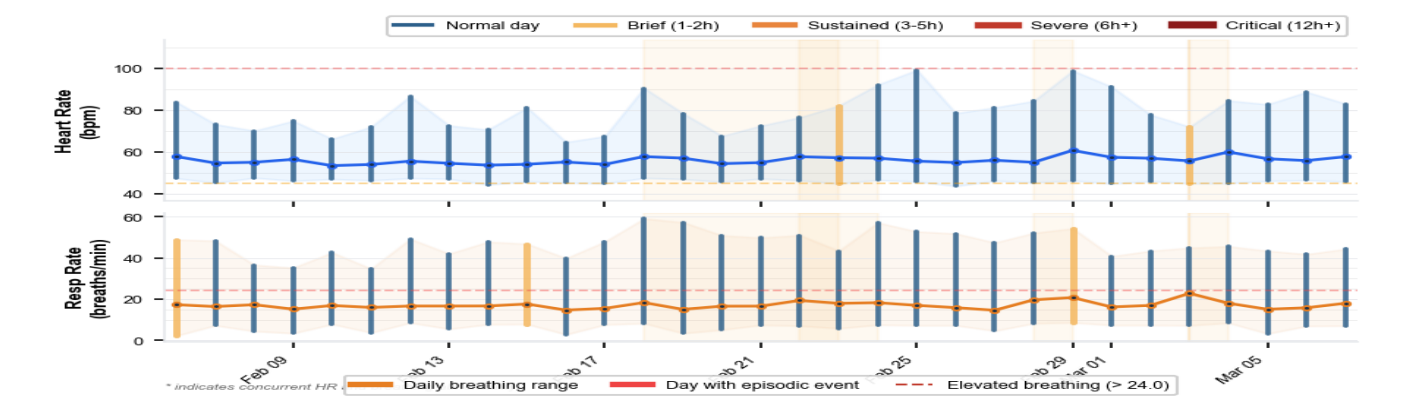
RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Elevated Breathing (avg 27 brpm, peak 57 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 31 brpm, peak 43 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Breathing (avg 41 brpm, peak 44 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 18 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

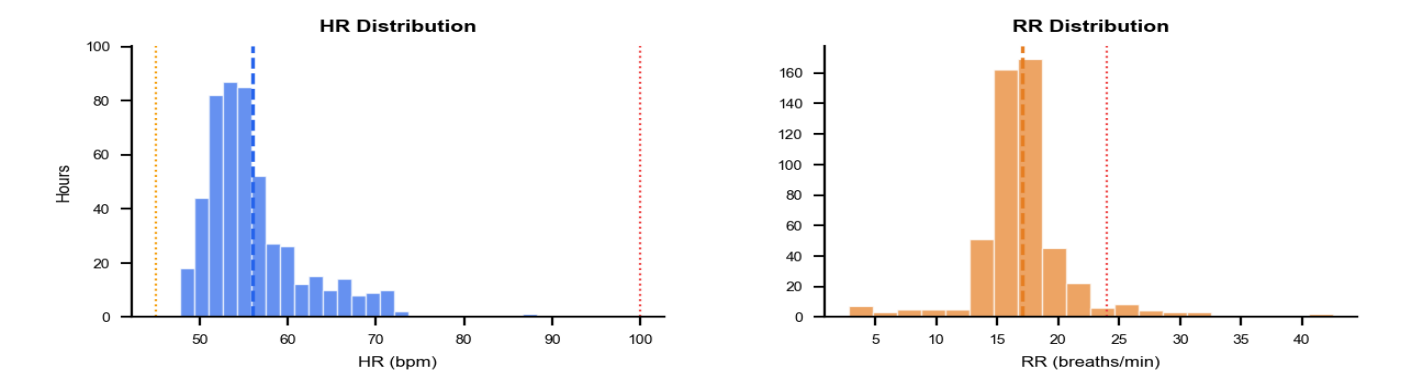
Clinical Pattern Observations:

- 1. Breathing variability: RR spread 10 brpm (12 to 23).
- 2. Clustered pattern: Episodic events on 5 of 31 days (16%).

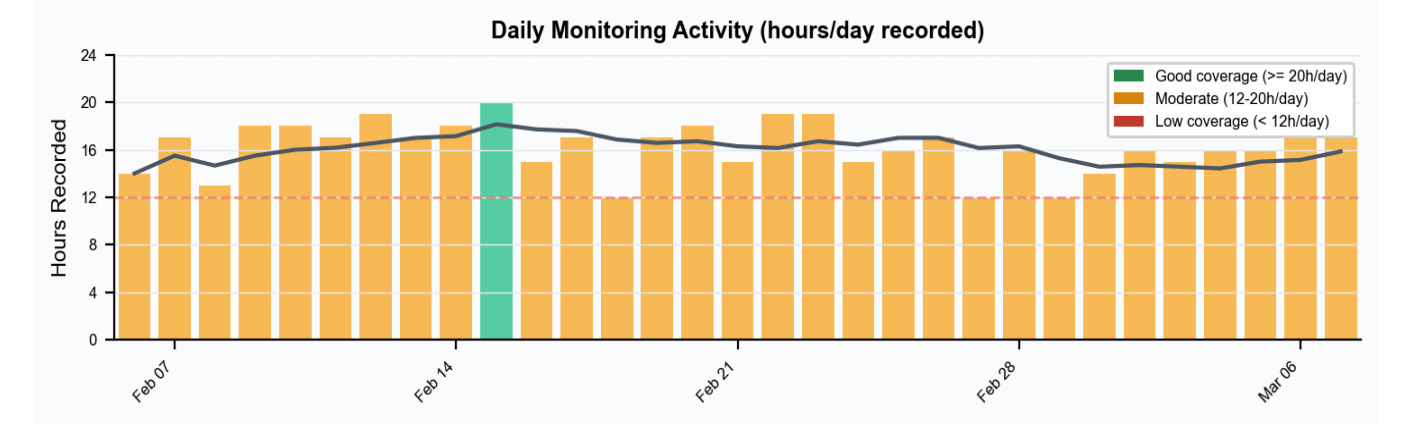


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for  $\geq 1$  hour. Episodes separated by  $\leq 1$  hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.